

How procedures are priced and who pays

A plain-language guide for AA directors, anaesthetists and the AA office

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Based on: Greg Smith's *Contract pricing model: design summary (v3)* and *Identification of the Billable Party* notes, revised after the 5 October 2026 review with Donald Fraser

Companion document: *Contract pricing model: technical design* (for the development team) covers the same rules in data and system terms

1. What this guide covers

Every procedure on a booking has to answer two questions before it can be billed:

- **What was done?** That is the **procedure**, for example a face-lift.
- **How is it paid for?** That is the **contract**: who has agreed to pay, and on what terms.

The new system keeps these two questions separate. The same face-lift can be paid for in different ways depending on the surgeon, the hospital, the insurer and the anaesthetist, but it is still the same procedure.

This guide explains how the system answers both questions, who receives the invoice, what the anaesthetist can change, and how prepaid procedures fit in. Section 10 lists the questions we need the directors to answer. The guide does not describe screens or databases. The technical design does that.

2. The building blocks

RVG groups

The NZSA Relative Value Guide (RVG) lists procedures in groups by body area, each with recommended **base units**. The system holds the RVG as published and uses it as the starting point for every procedure. AA can add its own groups if it ever needs to.

Procedures

A procedure is the specific operation, worded as it should appear on the invoice. Each procedure belongs to one RVG group and uses that group's base units unless AA sets a different figure for that procedure. The office finds a procedure by body area, then subgroup, then procedure, so the list stays short at each step.

Contracts

A contract is AA's record of how a procedure is paid for. It is not necessarily a legal contract. There are two kinds:

	Third-party contracts	The anaesthetist's own pricing
What it is	An agreement with someone else: an insurer (for example Southern Cross), a hospital, or a surgeon's rooms.	No contract (RVG): standard RVG pricing at the anaesthetist's own unit rate. Also any price list an anaesthetist keeps for their own work, for example for prepaid cosmetic procedures.
Price	Usually a fixed price per procedure, agreed for a period (often a year).	RVG units × the anaesthetist's own rate, or the anaesthetist's own price.

	Third-party contracts	The anaesthetist's own pricing
Can the anaesthetist change the price?	No. The agreed price stands.	Yes. The anaesthetist has full discretion.

“No contract (RVG)”

When nobody has a special arrangement, the business says there is “no contract” and the procedure is priced on the RVG. The system shows this as **No contract (RVG)**. It is available for every procedure and appears first in the list of choices.

Behind the scenes the system stores this as a contract like any other, so that every booking is priced the same way. Users never need to know that.

The key idea

The procedure says what was done. The contract says how it is paid for. One procedure can be paid for under many different contracts, and changing the contract never changes the procedure.

3. Who gets the invoice

The contract decides who is billed. Some contract holders pay AA themselves, such as an insurer that accepts direct claims. Others only set the price and the patient pays, as with a surgeon's fixed-fee arrangement for cosmetic work.

Situation	Contract chosen	Who is billed
An insurer or hospital has agreed to pay AA directly for this procedure	That insurer's or hospital's contract	The insurer or hospital
A surgeon has agreed a fixed price with AA, and the patient pays	The surgeon's fixed-price contract	The patient, or their parent or guardian
No arrangement, and the patient pays (including insured patients who claim the money back themselves)	No contract (RVG)	The patient, or their parent or guardian
The booking says an insurer will pay, but no insurer contract has been chosen	Not yet settled	Flagged for the office to check before billing

- When the patient pays, the invoice goes to the payer named on the booking. The patient's details are filled in automatically, and the office or anaesthetist can change them to a parent or guardian.
- The surgeon's booking often shows the patient's insurance status (for example SXAP), and the hospital often confirms it. That information guides which contract is chosen, but the contract is what decides who is billed.
- If the hospital cannot get insurance pre-approval on the day, the patient either goes home or pays. If they pay, the anaesthetist changes the contract to No contract (RVG) and the patient is billed. This happens regularly and the system supports it.

4. From booking to invoice

1. **The office sets up the booking.** It selects the procedure, then selects or confirms the contract. The list of contracts only shows those that apply to this hospital, surgeon and anaesthetist, with No contract (RVG) first.

2. **The anaesthetist reviews the booking (the card) in the mobile app.** They can change the procedure or the contract at any time until they submit the list. No explanation is needed, but the office sees the change at review. Changing either one refreshes the starting base units, modifiers or fixed price.
3. **After the procedure, the anaesthetist records base, time and modifier units (BTM).** Base and modifier units start from the procedure and contract, and the anaesthetist can change them. Any modifiers claimed need a short explanation, because insurers ask for one.
4. **The price follows from the contract.** Section 5 explains what the anaesthetist can and cannot change.
5. **The anaesthetist completes the booking and submits the list.** The office reviews it, corrects anything that needs correcting, and authorises it for billing.
6. **The system keeps a record of exactly what was used to price each procedure.** Later changes to the RVG or to a contract never change an invoice that has already been issued.

If a booking has more than one procedure, each one is priced on its own contract.

5. What the anaesthetist can change

	Third-party fixed price	Anaesthetist's own fixed price (for example a prepaid list)	No contract (RVG)
Where the price comes from	The contract's agreed price	The anaesthetist's own price for that procedure	BTM units × the anaesthetist's own unit rate
Role of BTM units	Recorded for reference only	Recorded for reference only	Used to calculate the price
Change the price	No. The price is shown but cannot be edited.	Yes, with a reason	Yes. The anaesthetist can type their own price, with a reason.
Percentage discount	No	Yes	Yes. A 100% discount or a price of \$0 gives a no-charge invoice.
Change the contract	Yes	Yes	Yes
Who is billed	Whoever the contract says: the insurer or hospital, or the patient for a surgeon's arrangement	The patient or guardian	The patient or guardian

The anaesthetist has the final say

AA acts as agent for independent anaesthetists. It is not their employer. The system lets the anaesthetist make the call, even when the office might disagree, and the office review is where mistakes are caught. The only limit is a price agreed with a third party. To depart from that price, the anaesthetist has to change the contract, which also changes who is billed.

The office can still correct any price at review, for example to fix a data-entry error.

6. Prepaid procedures

Some anaesthetists ask for certain procedures, usually cosmetic work paid for by the patient, to be paid in full before the procedure.

Already agreed

- Each anaesthetist chooses which procedures, or whole RVG groups, they want prepaid.
- Prepayment applies only when a person (the patient or a guardian) is paying, never an organisation.
- The full expected fee is prepaid. There is no part payment or deposit.
- The prepayment invoice is created when the booking is set up and sent once the office approves it. The money is held in trust until the procedure is done, and refunded if the procedure is cancelled.
- If the work turns out to be less than expected, nothing is refunded.

Proposed after the 5 October review (please confirm)

- The prepaid amount works like the anaesthetist's own fixed price. By default it is the final price, and the BTM units recorded on the day are for reference only.
- If the procedure took more than expected, for example because of a complication, the anaesthetist can raise the price. The difference is invoiced to the patient as an extra invoice.

Not yet decided

Where the prepaid amount comes from. Each anaesthetist must set their own prices, because AA cannot set a common price for its members. The two options are (a) each anaesthetist keeps their own price list for prepaid procedures, or (b) the system estimates the fee from expected units at the anaesthetist's own rate.

Whether the anaesthetist sees the prepaid amount on the card. Greg's view is that clinical work should be kept apart from billing work, but anaesthetists may prefer to see it.

7. Modifiers

Page 12 of the RVG gives guidance on when modifier units may be added to the base units, with typical and maximum values for each body area. They are optional and depend on the patient and the procedure. Vanessa also keeps a list of modifiers that AA uses which are not in the RVG.

Ben's view is that modifiers are not part of the standard schedule and should never be added automatically. The proposal is:

- The system never adds modifiers on its own.
- It can show the anaesthetist the RVG guidance that applies to the procedure, plus AA's own modifiers, so they can pick from a short, relevant list.
- Each modifier claimed needs a short explanation.

Earlier discussions proposed pre-filling modifiers from the patient's ASA grade and from defaults on each procedure. If the directors agree with Ben, that pre-filling would be dropped.

8. Worked examples

Example	Contract	Price	Billed to
Insured patient, insurer has a fixed-price agreement with AA for the procedure	Insurer's contract (third party)	Agreed fixed price. The anaesthetist cannot change it.	Insurer
Face-lift with a surgeon who has a fixed-fee arrangement with AA	Surgeon's contract (third party)	Agreed fixed price. The anaesthetist cannot change it.	Patient
The same face-lift with a surgeon who has no arrangement	No contract (RVG)	BTM × the anaesthetist's own rate	Patient

Example	Contract	Price	Billed to
Insurance pre-approval falls through on the day and the patient decides to pay	Anaesthetist changes it to No contract (RVG)	BTM × own rate, or the anaesthetist's own price	Patient
An anaesthetist who charges a flat price rather than using the RVG	No contract (RVG)	The anaesthetist types their own price	Patient
A colleague's family member, at a reduced rate	No contract (RVG)	50% discount, or \$0 for a no-charge invoice	Patient (a no-charge invoice)
Prepaid cosmetic procedure where a complication added time	Anaesthetist's own pricing	The prepaid amount, unless the anaesthetist raises it. Any increase is invoiced separately.	Patient

9. Keeping prices up to date

- The office maintains the RVG groups and the procedure list, and every change is recorded.
- Each contract has a start date and, if needed, an end date. A price review creates a new dated version of the contract. Bookings priced under the old version keep the old price.
- Contract holders send price reviews in different forms today, mostly PDFs and a few spreadsheets. We propose that all price lists and other master data come to AA in a standard spreadsheet. How those spreadsheets are loaded into the system (a one-off load followed by office editing, or an upload screen with a preview) is still to be agreed.

10. Questions for the directors

1. **Modifiers.** Should the system ever add modifier units automatically, or only show the RVG guidance and let the anaesthetist choose? Would the page 12 guidance be useful in the app?
2. **Prepaid amount.** Should each anaesthetist keep their own price list for prepaid procedures, or should the system estimate the fee from units at their own rate?
3. **After a prepaid procedure.** Is it right that by default nothing more is billed, that the anaesthetist can raise the price and have the difference invoiced, and that nothing is refunded if the work was less?
4. **Seeing the prepaid amount.** Should the anaesthetist see the prepaid amount on the card when recording their units?
5. **Contract terms.** Do all current contracts work on either RVG units or a fixed price per procedure? Is there any contract with a special unit rate or a percentage discount?
6. **Group prices.** Does any contract set one price for a whole RVG group rather than per procedure?
7. **Insurer but no contract.** If a booking says an insurer will pay but no insurer contract has been chosen, is a warning to the office the right response?
8. **Price reviews.** Would contract holders be willing to send price reviews in a standard AA spreadsheet?